



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : ايمان مطيع محمد ابراهيم
المحافظة : القاهرة
الحى / المدينة : البساتين
رقم البطاقة : 28708130103608 رقم التليفون : 01140739032
العنوان : -----
العمر (تاريخ الميلاد) : 13-08-1987 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 29-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام رنين مغناطيسي علي الفقرات العنقيه والظهيره
قسم العلاج :
الأورام [11108]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: إيمان مطيع محمد إبراهيم National ID: 28708T30103608	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي MRI of cervical and thoracic spine

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية



الاسم
مطيع محمد ابراهيم
الاسم
ابن احمد الوكيل
المستقر
القاهرة

١٩٨٧/٠١/٢٣

٢٨٧٠٨١٣٠١٤٦٠٨

IC3193206



٢٠١٩/٠٧ ٢٨٧.٨١٣.١٠٣٦.٨
حاصلة على بكالوريوس خدمة اجتماعية



أنشئ مبلغة متروجة
عبدالمعزم حسنين مختار حسنين
البطاقة سارية حتى ٢٠٢٦/٠٧/١٩





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : **امان بطيع محمد ابراهيم**

Sex:

MDT form

3- Medical number:

Female

83142908

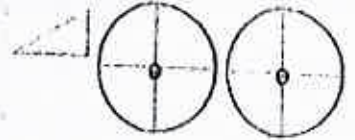
Old Med :

Old Med :

1810-2023

2-ID number: **28708130103608**

Patient Age: **36 Y**



Family history

Family history
+VE

Phone

Phone

01140739032

BMI

BMI

REASON OF MDT

REASON OF MDT
POST OPERATIVE

marital status

Marital status
MARRIED

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co morbidity

Co Morbidity
FREE

Menopausal
status

Menopausal status
PREMENOPAUSAL

Initial STAGE

INITIAL STAGE
AC-TNM

Date of MDT

Date of MDT

21/12/20

Clinical data					
Breast Imaging :			Mets Workup :		
			CTCAP-->FREE		
BX IDC BX COMMENT: GIII, TYPICAL MEDULLARY FEATURES	ER 7/8 ER COMMENT:	PR 2/8 PR COMMENT:	Her 2 Neu 0 Her 2 Neu COMMENT:	SISH SISH COMMENT:	KI67 50
T 2	N 1	M 0			
Sonomammography					
LT UOQ PROBABLY BENIGN LOOKING SOLID LESION WITH ATYPICAL FIBROADENOMA					
PATIENT HISTORY					
PT received 4 AC + 14 taxanes					
LT WLE+SLNB:IDC GIII with DCIS with focal microcalcification					
free margins , LN 0/4 , YPT2 NO					
MDT OUTCOME & PLAN					
FOR 1-PORT.					
2- ZOLADEX FEMARA.					
3- ZOMETA EVERY 6 MONTHS					

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY / /	RADIOLOGY	RADIOLOGY / /
SURGERY	SURGERY / /	PATHOLOGY	PATHOLOGY / /
Psychiatric	Psychiatric / /	Nutrition	Nutrition / /



Patient Name	: الأستاذ / ايمان مطيع محمد ابراهيم	Request No.	: 13002967
Patient No.	: 28708130103608	Request Date	: 09/05/2023
Path No.	: 1360/2023	Verification Date	: 23/05/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 35 Y	Department	: Out Patient
Referring Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 09/05/2023, 01:40 pm.

Gross pathology:

- Multiple grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for Immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma with atypical medullary features.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+3) Grade III.
- * Ductal carcinoma in situ: Absent
- * Perineural Invasion: Absent
- * Angiolinvasion: Absent
- * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 50 % of tumor cells (Allred score 7/8).
 - PR: Weak nuclear reaction in 1% of tumor cells (Allred score 2/8).
 - Her2: No membrane reaction in tumor cells (score 0).
 - Ki67: 50% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma with atypical medullary features, Grade III.

IPT:

- ER: Positive (score 7/8).
- PR: Negative (score 2/8).
- Her2: Negative (score 0).

Verified by :

Dr.Marwa mahmoud

Printing Date : 23/05/2023 10:02:57

Page 1 of 2

Printing By : Ali mostafa



Patient Name: Eman Motie Mohamed Ibrahim	Date : 18/01/2025
Exam : C.T.CHEST	Age : 37 Year(s)
Medical ID: 1810/2023	Pat. ID: 37751

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,

**** As Compared to previous study dated 11/05/2024, the current revealed:***

- ❖ Giving history of left BC.
- **Still noted** Left breast edematous parenchyma and thickened skin.
- **Still noted** Bilateral apical pleuropulmonary scarring with thick reticulations, likely of post granulomatous infection.
- **Still noted** Few bilateral upper lobar emphysematous bullae.
- **Newly developed** left upper lobe anterior segment and lingula subpleural fine reticulations with atelectatic bands, Likely post radiation sequel
- **Otherwise**, Clear both lung fields. No sizable suspicious pulmonary nodules, masses, areas of consolidation.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size, No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta
- Upper abdominal cuts are unremarkable

Opinion:

- No CT evidence of pneumonia or metastatic pulmonary disease.

Dr: Ahmed Elgazar

Dr. Walid Mohamed, M.D.



CAIRO SCAN
Radiology & Ultrasound



معمل لاب ميد إيجيبت
للطب والتشخيص
Labmed Egypt
Radiology & Ultrasound

Name : Mrs. Iman Moteaa Mohamed Ibrahim
Date : 17/06/2023

Code : R2907394

Tc-99m MDP BONE SCAN:

HISTORY:

- Female patient, with history of recently diagnosed breast cancer, referred for metastatic work up.

PROCEDURE:

- Whole body skeletal survey was obtained 3 hours post injection of 22mCi of the tracer.

FINDINGS:-

- No scintigraphic evidence of metastatic bone deposits could be detected all over the scanned skeleton.
- Degenerative changes of the spine are noted more pronounced at the lower lumbar and sacral vertebrae.
- Pronounced osteoarthritic changes of both shoulders and knees are also observed.

OPINION:

- No definite scintigraphic evidence of bone metastases.

Best regards

Dr. Esraa El-Kholy, MD.

Assist. Professor of Nuclear medicine (NCI)
Cairo University

For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg

19314

Mohandessen , Giza , Masr El Gedida , Nasr City , Shoubra

Mokattam , Downtown , Maadi , Helwan , Mantah , Fayssal , Zamalek

☎ 012 7220 6826

www.labmedegypt.com

Name: Mrs. Eman Motea Mohammed Ibrahim
Date: 2025-12-02

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Post left CBS and ALAND followed by radiotherapy, routine follow up study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- Thickened skin and coarsen trabeculae.
- UOQ architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.
- **Ultrasound** demonstrated UOQ area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
- Thickened skin reaching 5 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion: -

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 2): Post-therapeutic sequelae.

Recommendation:-

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	



د. م. س.
رئيس







Patient's Name: Iman Moti Mohammed Ibrahim
Date: 2026-08-19
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

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BIRADS 3: Probably benign, for close follow up.	2 %

Best Regards,
Dr Hanaa Abou Elmagd

skin mark were placed over the two wires

2- wire was inserted in sub cutaneous clip
cm below the skin.

1- Palpable malignant mass at the UOQ about 7 cm from the nipple and 1

LT breast:

at the site of concern in the:

Under complete sterile conditions, 3ml of local anaesthetic were administered

Technique:

centre of mass and other under skin

Clinical summary: left breast known malignant mass with two clip /one at

Mammographic Guided Stereotactic Wire localization:

Patient Name: Iman metaw Date: 26/11/2023



البرنامح الوطني للصحة العامة
وزارة الصحة والسكان

Presidential Program for Women's Health
Ministry of Health and Population





Echo report

مستشفى الامام دار السلام
الطبية المتخصصة
والتشخيصية

Name: *Abdulaziz*
Date: 8/6/2023

EF	64%	55-75%	PWT		0.6-1.1cm
LVEDD	3.8	3.5-5.8cm	AO.R	2.3	2-3.7cm
LVEDS	2.5	2-4cm	LA	2.3	2-4cm
FS	34	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions

LV wall thickness

LV diastolic function

SWMA No Pericard at rest.

LA and Aortic root dimensions

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion

No

D/ Mohamed Ali

Msc cardiology

Dr. Nourhan Omar
Dr. Amr Farouk M.D.

- No pulmonary or hepatic metastatic deposits on non contrast basis.

Opinion:

- Right iliac bone tiny sclerotic lesion likely bone island.
- No abdominal or pelvic masses or collections.
- Normal appearance of the other pelvic organs.
- Diverticular outpouching.
- Smooth filling of the urinary bladder with no filling defects or Normal appearance of the pancreas, aorta, I.V.C.
- Normal size, parenchymal thickness & density of both kidneys (apart from tiny left renal upper calyceal stone) with no back-pressure changes.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & contour irregularities.
- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.

II Abdomen & pelvis:

- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal cardiac size, No pericardial effusion detected.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No hilar or mediastinal lymph nodes.
- Few bilateral upper lobar emphysematous bullae.
- masses, areas of consolidation.
- Otherwise, Clear both lung fields. No sizable pulmonary nodules.
- Bilateral apical reticulations likely of post granulomatous infection.
- Left breast edematous parenchyma and thickened skin.

I Chest:-

- ❖ Compared to previous study dated 24/5/2023
- ❖ Giving history of left BC.

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:-

Medical ID:	Patient Name: Eman Motie Mohamed Ibrahim
Exam: C.T CHEST	Date: 11/05/2024
	Age: 36 Year(s)
	Pat. ID: 37751

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



مركز التخصص في السرطان
دار السلام
قسم الأشعة
التصوير المقطعي المحوسب

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Right iliac bone tiny sclerotic lesion likely bone island.

II Abdomen & pelvis:

- Left breast UOQ enhancing lesion measures 1.8 x 2 cm with prominent left axillary lymph nodes.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

I Chest:-

Findings:

CHEST, ABDOMEN, AND PELVIS: -

POST CONTRAST MULTI-SLICE CT SCAN OF THE

Patient Name: Eman Motie Mohamed Ibrahim	
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	
Medical ID:	
Date : 24/05/2023	Age : 35 Year(s)

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة
مركز التخصص في
السرطان دار السلام
قسم الأشعة
التصويرية

وزارة الصحة والسكان
مركز الأشعة
مستشفى أبو حماد بن المنصور
الطريق 51 الطبية الجديدة
الجيزة 12511



Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

Patient Name: Eman Motie Mohamed Ibrahim	
Exam : ultrasound BREAST (BOTH SIDE)	Date : 31/08/2023
Age : 36 Year(s)	
Medical ID: 1810/2023	

HIGH RESOLUTION ULTRASOUND EXAMINATION OF BOTH BREASTS

Indication:

Pathologically prove left breast carcinoma under neoadjuvant chemotherapy.

Report:

- Left breast ill defined spiculated hypoechoic mass lesion, with internal vascularity detected within on color Doppler, seen at 2 to 3 o'clock position 0.3 cm from the overlying skin, measuring 2.6x1.8 cm at today's study comparing to 1.9x1.6 cm at the prior study, associated with another anterior well defined irregular small hypoechoic solid nodule located at 3 o'clock measuring about 0.5x0.3 cm.
- No suspicious solid or cystic lesions on the right breast.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific axillary LNs.

Opinion:

- Left breast progressive course of the UIC pathologically proven malignant mass lesion with another anterior newly developed suspicious nodule for further contrast study to exclude multifocality and multicentricity (BIRADS 6)
- Normal study of right breast (BIRADS 1)

Yours truly

Dr/Asmaa Zeinhom, Msc

Not Approved

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
مركز التخصص في علاج السرطان
مستشفى دار السلام
إدارة الأشعة

Patent Name: Eman Motie Mohamed Ibrahim	Date : 12/10/2023
Exam : MAMMOGRAPHY BOTH	Age : 36 Year(s)
Medical ID: 1810/2023	Pat. ID: 37751

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND

Indication:

Post NACT for left breast cancer

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left UOQ irregular dense lesion with clips seen inside another clips seen outside the mass corresponding by US to :
- Regressive course of the previously seen left breast ill defined spiculated hypoechoic mass lesion, with internal vascularity detected within on color Doppler, seen at 2 to 3 o'clock, measuring 2.1 x 1.5 cm compared to 2.6x1.8 cm at the prior study.
- Stationary another anterior well defined small hypoechoic solid nodule located at 3 o'clock measuring about 0.5x0.3 cm.
- No right breast suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- No suspicious solid or cystic lesions on the right breast.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific axillary LNs.

Opinion:

- Left breast regressive course of the UOQ pathologically proven malignant mass lesion (BIRADS VI).
- Stationary course of the left 3 o'clock small well defined nodule.
- Normal study of right breast (BIRADS 1)

Yours truly
Dr/Ali Saber, Msc.

Request No.	: 13002967
Request Date	: 09/05/2023
Verification Date	: 23/05/2023
Financial Category	: Out Patient
Department	: marwa mohamed yousse
Registered by	

Final Data: Left breast mass.
Date of Specimen: Tru-cut biopsy, collected at 09/05/2023, 01:40 pm.

Pathology:

Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA kit for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-30).

Macroscopic Examination:

1. Histological type: Invasive duct carcinoma with atypical medullary features.
2. In situ carcinoma: Absent
3. Invasive: Absent
4. In situ carcinoma: Absent
5. Strong nuclear reaction in 50 % of tumor cells (Allred score 7/8).
6. Weak nuclear reaction in 1% of tumor cells (Allred score 2/8).
7. No membrane reaction in tumor cells (score 0).
8. 50% nuclear reaction of tumor cells.

Final mass, Tru-cut biopsy:

Invasive duct carcinoma with atypical medullary features, Grade III.

1. Positive (score 7/8).
2. Negative (score 2/8).
3. Negative (score 0).

Verified by:

Dr. Marwa mahmoud

Page 1 of 2

Printing By : Ali mostafa



Verified by :
Dr. Heba Hasan

Clinical data: Left breast cancer (post NACT)
Nature of specimen: Left WLE , SLNB
Gross examination:
• Procedure: Left WLE
Fibrotically tissue measured 8x6.5x2.5 cm
• Tumor extent : A wired and clipped greyish white tumour bed site
• Size: 2.5x2x1.5 cm .
• Distance from margins: 0.1 cm away from least margin (superior margin) 0.2 cm inferior margin, 2.5 cm from medial margin, 2.5 cm from lateral margin, 1.5 cm from deep margin, 2 cm from superficial margin
• Lymph nodes : 4 LNs up to 0.5 x0.5 cm
• Extra superior margin: fibrotically tissue piece 8x4x1.5 cm, free of gross induration
• Frozen section: Free margins , Free SLN (0/4)(superior margin is close)
Microscopic examination:
• Histological type: Invasive duct carcinoma (Cellularity 70%) with moderate necrosis
• Grade (differentiation+ nuclear pleomorphism + mitosis): (G III)(3+3+2)
• Ductal carcinoma in situ: Yes (5%)
• Lymph nodes : Free sentinal lymph nodes (0/4)
• Resection margins: Free margins , free extrasuperior margin
• Histologic response to therapy: Pathologic Partial Response
Diagnosis:
Left WLE & SLNB (post NACT) :
Invasive duct carcinoma grade III with DCIS with focal microcalcification
• Free margins
• Free sentinal lymph node (0/4)
• Pathologic Partial Response (PPR)
• Stage : ypT2, ypN0
• RCB : 2.215 , RCB class II

Pathology Report

Patient Name	الاسم / اسم المريض	Request No.	13006618
Patient No.	28708130103608	Request Date	27/11/2023
Path No.	4205/23	Verification Date	12/12/2023
Sex	Female	Financial Category	المرأة - فئة المرأة
Age	36 Y	Department	Out Patient
Referring Doc :		Registered by	marwa mohamed yous:





Presidential Initiative for Women's Health - Pathology Lab
الهيئة العامة للصحة العامة - وزارة الصحة - الكويت

Pathology Report	
Request No. : 13002967	Patient Name : اسم المريض / اسم المريض
Request Date : 09/05/2023	Patient No. : 28708130103608
Verification Date : 23/05/2023	Path No. : 1360/2023
Financial Category : نوع الفحص - نوع الفحص	Sex : Female
Department : Out Patient	Age : 35 Y
Registered by : marwa mohamed youss	Referring Doc :

Clinical Data: Left breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 09/05/2023, 01:40 pm.

Gross pathology:
- Multiple grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma with atypical medullary features.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+3) Grade III.
* Ductal carcinoma in situ: Absent
* Perineural invasion: Absent
* Angiolymphovascular invasion: Absent
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 50 % of tumor cells (Allred score 7/8).
- PR: Weak nuclear reaction in 1% of tumor cells (Allred score 2/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 50% nuclear reaction of tumor cells.

Diagnosis:
Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma with atypical medullary features, Grade III, IPT.

- ER: Positive (score 7/8).
- PR: Negative (score 2/8).
- Her2: Negative (score 0).

Verified by :
Dr.Marwa mahmoud



م/فان
٥/٢٤
فتح طه سنيني



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٢٢



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A- Patient ID:

1- Name: هبة منقح محمد ابراهيم

Sex:

3- Medical number:

Female

K3142808

Old Med:

18/07/2023

2- ID number: 28708130403608

Patient Age: 36 Y



Family History

POSITIVE F.H
HER MOTHER
ENDOMETRIUM
AND BREAST

Phone:

BMI

REASON OF MDT

NLW

marital status

NRS Score

GROUP

B

Date of MDT

2023-06-01

Co morbidity

FREE

Menopausal
Status

MENOPAUSE

Clinical data

Breast Imaging:

Mets Workup:
CTCAP → FREE

BX:

IDC

ER:

7/8

PR:

2/8

Her 2 Neu:

0

SISH:

0

K167:

50

GJT + ATYPICAL
MEDULLARY
FEATURES

T

2

N

1

M

0

Sonomammography

LT UOQ PROBABLY BENIGN LOOKING SOLID LESION WITH ATYPICAL FIBROADENOMA

PATIENT HISTORY

1- ECHO, LABS

2- BONE SCAN

3- CLIPPING

MDT OUTCOME & PLAN

FOR ZOLADEX+ START NACT ACX4 TAXOTEREX4 FOLLOWED BY LT BCS AND AXILLA ACC TO RESPONSE

SIGNATURE

MEDICAL ONCOLOGY

Date: 01/08/23

RADIOLOGY

Date: 01/08/23

SURGERY

Date: 01/08/23

PATHOLOGY

Psychiatric

Nutrition



Patient Name: iman metaw **Date:** 26/11/2023

Mammographic Guided Stereotactic Wire Localization:

Clinical summary: left breast known malignant mass with two clip /one at centre of mass and other under skin

Technique:

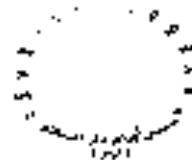
Under complete sterile conditions, 3ml of local anaesthetic were administered at the site of concern in the:

LT breast:

- 1- Palpable malignant mass at the UOQ about 7 cm from the nipple and 1 cm below the skin.
- 2- **wire was inserted** in sub cutaneous clip

skin mark were placed over the two wires

Best Regards,
Dr Hanaa Abou Elmageed



Patient Name: Eman Motie Mohamed Ibrahim	Date : 24/05/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 35 Year(s)
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast UOQ enhancing lesion measures 1.8 x 2 cm with prominent left axillary lymph nodes.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Right iliac bone tiny sclerotic lesion likley bone island.



Patient Name: Eman Motie Mohamed Ibrahim	Date : 24/05/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 35 Year(s)
Medical ID:	

Opinion:

- Left breast neoplastic lesion.
- No pulmonary or hepatic metastatic deposits.

Dr. Aya Deabes, FRCR

Dr. Oba Magdy, M.D.



Patient Name: Eman Motie Mohamed Ibrahim	Date : 31/08/2023
Exam : ultrasound BREAST (BOTH SIDE)	Age : 38 Year(s)
Medical ID: 1810/2023	

HIGH RESOLUTION ULTRASOUND EXAMINATION OF BOTH BREASTS

Indication:

Pathologically prove left breast carcinoma under neoadjuvant chemotherapy.

Report:

- Left breast ill defined spiculated hypoechoic mass lesion, with internal vascularity detected within on color Doppler, seen at 2 to 3 o'clock position 0.3 cm from the overlying skin, measuring 2.6x1.8 cm at today's study comparing to 1.9x1.6 cm at the prior study, associated with another anterior well defined irregular small hypoechoic solid nodule located at 3 o'clock measuring about 0.5x0.3 cm.
- No suspicious solid or cystic lesions on the right breast.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific axillary LNs.

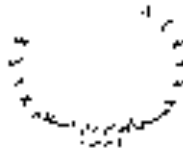
Opinion:

- Left breast progressive course of the USQ pathologically proven malignant mass lesion with another anterior newly developed suspicious nodule for further contrast study to exclude multifocality and multicentricity (BIRADS 6)
- Normal study of right breast (BIRADS 1)

Yours truly

Dr//asmaa Zainhom, MSc

Not Approved



Patient Name: Eman Motie Mohamed Ibrahim	Date : 12/10/2023
Exam : MAMMOGRAPHY BOTH	Age : 36 Year(s)
Medical ID: 1810/2023	Pat. ID: 37751

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Post NACT for left breast cancer

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left UOQ irregular dense lesion with clips seen inside another clips seen outside the mass corresponding by US to :
- **Regressive course** of the previously seen left breast ill defined speculated hypoechoic mass lesion, with internal vascularity detected within on color Doppler, seen at 2 to 3 o'clock, measuring 2.1 x 1.5 cm compared to 2.6x1.8 cm at the prior study. **3.0 x 2.0**
- **Stationary** another anterior well defined small hypoechoic solid nodule located at 3 o'clock measuring about 0.5x0.3 cm.
- No right breast suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- No suspicious solid or cystic lesions on the right breast.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific axillary LNs.

Opinion:

- **Left breast regressive course of the UOQ pathologically proven malignant mass lesion (BIRADS VI).**
- **Stationary course of the left 3 o'clock small well defined nodule.**
- **Normal study of right breast (BIRADS 1)**

Yours truly

Dr/Ali Saber, Msc.

Name : الأستاذ / أيمن مطيع محمد إبراهيم	Request No. : 13002967
ID No : 28708130103608	Request Date : 09/05/2023
No. : 1360/2023	Verification Date : 23/05/2023
Sex : Female	Financial Category : نفقة شوية - ارقام دار السلام
Age : 35 Y	Department : Out Patient
Ref Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.

Site of Specimen: Tru-cut biopsy, collected at 09/05/2023, 01:40 pm.

Pathology:

1. Grayish white tissue cores up to 1 cm, totally submitted.

2. Slides prepared from paraffin block for immune-staining using Bench Mark ULTRA

3. Staining for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-

4. Microscopic Examination.

5. Histological type: Invasive duct carcinoma with atypical medullary features.

6. Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+3) Grade III.

7. In situ carcinoma: Absent

8. Lymphovascular invasion: Absent

9. Perineural invasion: Absent

10. Immunohistochemistry (Positive control showed appropriate reactivity).

11. ER: Strong nuclear reaction in 50 % of tumor cells (Allred score 7/8).

12. PR: Weak nuclear reaction in 1% of tumor cells (Allred score 2/8).

13. Her2: No membrane reaction in tumor cells (score 0)

14. Ki67: 50% nuclear reaction of tumor cells.

15. Interpretation:

16. Left breast mass, Tru-cut biopsy:

17. Invasive duct carcinoma with atypical medullary features, Grade III.

18. ER: Positive (score 7/8).

19. PR: Negative (score 2/8).

20. Her2: Negative (score 0).

Verified by :

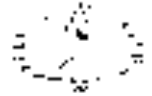
Dr. Marwa mahmoud

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Page 1 of 2

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المبادرة الرئاسية لصحة المرأة - معمل تشخيصي
Presidential Initiative for Women's Health - Pathology Lab



Patient Name	: الأمتة / ايمان مطيح محمد ابراهيم	Request No.	: 13006618
Patient No.	: 28708130103608	Request Date	: 27/11/2023
Path No.	: 4205/23	Verification Date	: 12/12/2023
Sex	: Female	Financial Category	: ارقام دار السلام - نفقة التركة
Age	: 36 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical data: Left breast cancer (post NACT)

Nature of specimen: Left WLE , SLNB

Gross examination:

• Procedure: Left WLE

Fibrofatty tissue piece measured 8x6.5x2.5 cm

• Tumor extent : A wired and clipped greyish white tumour bed site

• Size: 2.5x2x1.5 cm .

• Distance from margins: 0.1 cm away from least margin (superior margin) 0.2 cm inferior margin, 2.5 from medial margin, 2.5 cm from lateral margin, 1.5 cm from deep margin, 2 cm from superficial margin

• Lymph nodes : 4 LNs up to 0.5 x0.5 cm

• Extra superior margin: fibrofatty tissue piece 8x4x1.5 cm, free of gross induration

• Frozen section: Free margins , Free SLN (0/4)(superior margin is close)

Microscopic examination:

• Histological type: Invasive duct carcinoma (Cellularity 70%) with moderate necrosis

• Grade (differentiation+ nuclear pleomorphism + mitosis): G III (3+3+2)

• Ductal carcinoma in situ: Yes (5%)

• Lymph nodes : Free sentinal lymph nodes (0/4)

• Resection margins: Free margins , free extrasuperior margin

• Histologic response to therapy: Pathologic Partial Response

Diagnosis:

Left WLE & SLNB (post NACT) :

Invasive duct carcinoma grade III with DCIS with focal microcalcification

• Free margins

• Free sentinal lymph node (0/4)

• Pathologic Partial Response (PPR)

• Stage : ypT2, ypN0

• RCB : 2.215 , RCB class II

Verified by :

Dr.Heba Hasan

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Page 1 of 2

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